

PRIORITIZATION, DELEGATION AND STAFF MANAGEMENT

PRIORITIZATION

You need to... decide which patient is sickest or healthiest.

- Be sure you know which one you're looking for

EXAMPLES:

- There's been a disaster in your town and you have to discharge one of the following people to make room for incoming wounded, which patient would you discharge? Highest priority or lowest priority? **LOWEST**
- You receive report on all of the following four patients, which patient will you check first before you get out of report? Highest priority or lowest priority? **HIGHEST**

The answer will always have four parts.

- **Age**
- **Gender**
- **Diagnosis**
- **Modifying phrase**

EXAMPLES:

10-year-old male with hypospadias whose throwing out bile stained emesis.

*** Two of those are always irrelevant and you do NOT use them in your answer, EXCEPT in pediatric question.**

- Age
- Gender

*** Diagnosis and the Modifying phrase are both important. Which is MORE important?**

- Modifying phrase is always more important

EXAMPLES:

- The patient had angina pectoris versus myocardial infarction; who is the higher priority patient? **MI.**
- The patient with angina pectoris + with unstable VS versus MI with stable VS? **Angina pectoris with unstable VS.**

THIS IS WHY YOU CANNOT USE ABC'S!

THE RULES FOR PRIORITIZATION

1. ACUTE BEATS CHRONIC

Example: If you have the following patients, who is the highest priority?

- a. CHF
- b. COPD
- c. Appendicitis

Appendicitis, because it's acute versus the others which are chronic.

If you used the ABC's → COPD → WRONG!

2. FRESH POST-OP (FIRST 12 HOURS) BEATS MEDICAL OR OTHER SURGICAL

Example: Who is your highest priority patient?

- a. COPD
- b. Acute Appendicitis
- c. CHF
- d. 2 hr post-op cholecystectomy
- e. 2 day post-op CABG
- f. Radical neck dissection
- g. Bilateral above the knee amputation
- h. Right frontal craniotomy

2 hr post op cholecystectomy, because they are FRESH post op.

Doesn't matter how bad the surgery is, if you aren't 12 hours in, you aren't high risk. You are recovery!

3. UNSTABLE BEATS STABLE

Words to look for in an answer or modifying phrase that makes a patient either:

<u>STABLE</u>	<u>UNSTABLE</u>
"Stable"	"Unstable"
Chronic Illness	Acute Illness
Post-op > 12 hours	Post-op < 12 hours
Local or Regional anesthesia	General anesthesia but only in first 12 hrs
Lab abnormalities of an A or B level	Lab abnormalities of a C or D level
"Ready for discharge" "To be discharged" "Admitted longer than 24 hours ago"	"Not ready for discharge" "Newly admitted" "Newly diagnosed" "Admitted less than 24 hours ago"
Unchanged assessments	Changing or changed assessments
Experiencing the typical expected signs and symptoms of the disease with which they were diagnosed	Experiencing unexpected signs and symptoms of the disease with which they were diagnosed

MISTAKE: Do not prioritize based on symptom severity. Prioritize on appropriateness of symptom occurrence.

Example:

One patient experiencing mild pain versus a patient experiencing severe pain.

If the patient has severe colick pain who has kidney stones → This is EXPECTED.

If the patient has mild pain who had a CXR → This is UNEXPECTED.

Therefore, mild pain is unstable!

Learning check → APPLICATION OF THE RULES:

1. Who is the higher priority?

- a) 16 year old female with meningococcal meningitis who has had a temperature of 103.8F (39C) since admission 3 days ago.
- b) 67 year old male with IBS who spiked a temperature of 100.3F (37.9C) this afternoon.

Thought process:

OPTION A	OPTION B
Rule out age and gender	
1. Meningitis = Acute (^)	1. IBS = chronic (v)
2. No fresh post op	2. No fresh post op
3. "who has had" = stable (v) "fever" = expected (v) "admission 3 days ago" = stable (v)	3. "spiked" = unstable (^) "fever" = unexpected (^) "this afternoon" = unstable (^)
B has more ^ than A and they are in the MODIFYING PHRASE. B is the answer.	

FOUR THINGS THAT ARE ALWAYS UNSTABLE REGARDLESS OF WHETHER IT IS EXPECTED OR NOT:

1. Hemorrhage

- DIC and hemophilia. He's hemorrhaging.
 - Don't say that you would expect a pt with DIC and hemophilia to hemorrhage, so since it's expected, it's low priority. NO, this is a medical emergency!
 - Don't confuse hemorrhage with bleeding. There's a huge difference.
 - "Bleeding" = could be high or low depending on if it's an expected symptom
 - "Hemorrhaging" = HIGH

2. High fevers over 105F/40C

- Even if it's expected because they will SEIZE.

3. Hypoglycemia

- A patient has hypoglycemia. Their BGL is 3.0. Even though this is expected, it's a HIGH priority.

4. Pulselessness or Breathlessness

- Pt with vtach/asystole has no pulse. Even though this is expected, it's a HIGH priority.
- **Except** at the scene of an unwitnessed accident → they are dead → LOWEST priority
 - BUT if you WITNESS the accident → they are the HIGHEST priority

THREE THINGS THAT RESULT IN A BLACK TAG IN AN UNWITNESSED ACCIDENT:

If they have ANY of the three, you gotta go to someone else because they need your help more.

1. Pulselessness
2. Breathlessness
3. Fixed and dilated pupils (even if they are breathing and have a pulse)

4. CAUTION: ONLY USE THIS AS A TIE BREAKER

The more vital the organ, the higher the priority. It is the organ of the MODIFYING PHRASE!

ORDER OF ORGAN VITALITY

1. Brain
2. Lung
3. Heart
4. Liver
5. Kidney
6. Pancreas

Example:

- a. CHF with K of 6.6, no EKG changes.
- b. CRF with creatinine of 24.7 with pink frothy sputum.
- c. Acute hepatitis with jaundice, increase ammonia level who you cannot arouse
- d. Angina pectoris with crushing chest pain not relieved by rest or 3 nitroglycerin

OPTION	ACUTE VS CHRONIC	STABLE VS UNSTABLE	ORGAN SYSTEM
A	v	^ (K level)	Heart
B	v	v (expected) ^ (unexpected)	Lung NOT kidney because of the pink frothy sputum
C	^	v (jaundice expected) v (ammonia level) ^ (LOC unexpected)	Brain NOT liver
D	V	v (expected) ^ (unexpected)	Heart
Therefore, C wins because of the tie breaker!!!			

DO NOT DELEGATE THE FOLLOWING RESPONSIBILITIES TO AN LPN

1. STARTING an IV.
2. Hanging or mixing IV medications.
3. Pushing IV PUSH medications. They can MAINTAIN and DOCUMENT the flow.
4. Administer blood or mess with central lines (flushing or even changing a dressing).
5. PLAN care. The RN makes the care plan. The LPN can implement it.
6. Perform or develop teaching. They can REINFORCE teaching.
7. Care for UNSTABLE patients.
8. The FIRST of anything because the RN must do this in order to AX and PLAN.
 - a. Can they tube feed? Yes. But not the first one after an NG insertion.
 - b. Can they change post op dressings? Yes. But not the first one.
 - c. Can they ambulate post op clients? Yes. But not the very first time getting out of bed.
 - d. Can they take VS? Yes. But not the first set of VS post-op sx.
9. Not allowed to do the following assessments:
 - a. Admission
 - b. Discharge
 - c. Transfer
 - d. The first one after there has been a change in status

Example:

In report, a nurse says, "I think I heard crackles in the patient. He's never had that before..."
Therefore, the RN should assess the patient because something may have changed.

Who should the LPN check?

- a. Pt with angina pectoris with substernal chest pain admitted 3 days ago on nitro
- b. Pt with subtotal thyroidectomy, two days ago, who says "Why are they washing elephants in the parking lot?"

LPN should check A. RN should check B. He could be going into a thyroid storm.

DO NOT DELEGATE THE FOLLOWING RESPONSIBILITIES TO A PSW

1. Charting.
 - a. They can chart about what they did but not about the patient.
2. Give medications except for TOPICAL OTC BARRIER CREAMS.
 - a. Nitroglycerin ointment? NO.
 - b. Neosporin ointment? NO, it's not a barrier cream.
 - c. Hydrocortisone? NO, it's not a barrier cream.
 - d. Elace ointment for the skin? NO, it's an enzyme.
3. Assessments except for VITALS and ACCUCHECKS.
4. Treatments, EXCEPT FOR ENEMAS.
 - a. Be very cautious when allowing them to catheterize.
 - i. If you could pick any other answer than catheterize, pick the other catheterize.
 - ii. But if it says catheterize a pt, IV, intubate, irrigate... Then catheterize is the answer.
 - b. Empty bag on an ileal conduit pouch versus evaluating the effects of ROM
 - i. Empty the bag because CANNOT evaluate
 - ii. Look at the VERBS

YOU MAY DELEGATE TO AIDS:

1. ADLs but NEVER the first

Note: IN an EXTENDED CARE FACILITY, LPNs can do many of the things they cannot do in a hospital because the clientele is considered STABLE.

Do NOT delegate to the family SAFETY responsibilities.

- If a family member says, "Can you leave the restraints off my dad while I'm here and when I leave, you can put them back?" **NO!**
- What about a sitter? **YES.**
 - To be designated a sitter, you have to TEACH them and DOCUMENT that you have taught them. They can only do what you teach them to do.
- **Example:** If MD decides that the mother can give the kid an insulin shot. Can you do it? YES. As long as you document in the chart that you taught her and ensured that she is competent in the skill.
- **Example:** What if mother says, "I'm not done finishing my baby yet. Would you leave the side rail down and I'll finish bathing her and I'll put it back up when I'm done." NO. "I'll stay with you to help you with this bath and when you're finished I'll put the side rail up."

STAFF MANAGEMENT

HOW DO YOU INTERVENE WITH INAPPROPRIATE BEHAVIOUR OF STAFF?

There are always four answers:

1. Tell supervisor.
2. Confront them and take over immediately.
3. At a later date, just talk to them about it.
4. Let it go/ignore it.

One of those is never the answer: IGNORE. You never ignore the behaviour. You always take it as an opportunity to teach.

How to choose between the remaining 3 options:

1. Is what they are doing illegal?

- a. If the answer is YES → Always choose 1: Tell supervisor.
- b. If the answer is NO...go to number 2

2. Is anyone (patient/staff member) in immediate physical or psychological harm?

- a. If the answer is YES → choose 2: Confront them and take over immediately.
Don't choose 1 because delaying confronting in order to tell supervisor would put the patient at risk. You may have to tell them later on but you need to deal with it right then.

If it's illegal and harmful, first you intervene and then you tell supervisor.

- b. If it's NO → go to number 3

3. Is this behaviour legal, not harmful, but simply inappropriate?

- a. If the answer is YES → choose 3: Counsel them later on. No hurry. No rush.

Examples:

1. You suspect that an RN with whom you work is diverting narcotics for private use?

- a. Illegal so tell supervisor.

2. You walk by the room of an AID who is giving perineal care to a patient and the AID is not wearing gloves.

- a. Not illegal.
- b. Anyone in harm? AID.
- c. Confront them and take over.

3. You notice an RN going home with bulging pockets.

- a. Illegal → stealing
- b. Tell supervisor.

4. You are an LPN in the OR and you notice the surgeon contaminates the pinky of the left hand.

- a. Not illegal.

- b. Anyone in harm? YES the patient
- c. Confront them.

5. RN giving report at shift, “Exasperation” instead of “exacerbations” of COPD.

What do you do?

- a. Not illegal.
- b. Anyone in harm? No.
- c. Talk about it later.

NOTE: When confronting somebody, you use “I” not “YOU” language.

6. Passing narcotics to patients but you notice he is somewhat incoherent and not as attentive as usual.

- a. Could be illegal.
- b. Anyone in harm? Yes.
- c. Confront and then report.

PEDIATRIC TEACHING

Review of Piaget's Cognitive Development Theory

They never mention Piaget's name but will ask about how you will teach children

FOUR STAGES FOR KID'S THINKING

STAGE	ORIENTATION	TEACHING
0-2 YEAR OLDS SENSORY MOTOR THINKING	Totally PRESENT oriented	<u>Teach them WHILE YOU DO IT.</u> Not ahead of time. And not after you've taught them. <u>What do you teach them?</u> What you ARE DOING. <u>How?</u> Verbally. Don't use play. <u>Pre-teach?</u> Parent
3-6 YEAR OLDS PRE- OPERATIONAL "PRE SCHOOL / PRE OPERATION"	Fantasy oriented Imaginative Illogical Thinking obeys NO rules You cannot reason with them They understand the future and the past	<u>Teach them BEFORE</u> <u>How long before?</u> Shortly before. Example: "The morning of, the day of, 2 hours before" <u>What do you teach them?</u> What you are going to do. Future tense. <u>How?</u> Play.
7 – 11 YEAR OLDS CONCRETE OPERATIONAL "711 is surrounded by concrete. No flowers"	Rule oriented Live and die by the rules They cannot abstract Rigid Only ONE way to do things	<u>Teach them DAYS AHEAD</u> <u>How long before?</u> A day or two before. <u>What do you teach them?</u> What you are going to do AND how to do skills. They will do it the way you taught them every single time. <u>How?</u> Age appropriate reading and demonstration.
12 – 14 YEAR OLDS	Abstract * Think CAUSE → EFFECT	Can now MANAGE their own care. Example: Which of the following can

FORMAL OPERATIONAL	NO LONGER A PEDIATRIC QUESTION; IT IS AN ADULT QUESTION. TEACH LIKE AN ADULT!	manage their own care? a) 8 yr old with cystic fibrosis b) 8 year old DM c) 10 yr old with a scraped knee d) 13 yr old with CRF D, yes!!! It's not the SEVERITY of the illness, it's the AGE in the kid. A 10 yr old cannot manage the scrape because you have to teach them how to wash it, polysporin, and bandage. Will do it the same way for as long as they are told to. What will they do when it's swollen? With pus? They will STILL DO IT. Managing doesn't mean doing everything it means knowing what they can and cannot do. Managing = 12+ Skills = 7+
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CHILDREN'S TOYS

When you select toys, there are usually 3 things to consider:

1. **Is it safe?**
2. **Is it age appropriate?**
3. **Is it feasible?**

SAFETY CONSIDERATIONS

1. No small toys under age 4 → RF aspiration
 - a. If they're over 4, it is fine
2. No metal toys if oxygen is in use
 - . RF sparks
- a. Fancy word that may be used → DIE-CAST
3. Beware of fomites
 - . Fomites → non living object that harbours micro organisms
- a. Vector → living thing that harbours micro organisms
- b. Toys are notorious fomites in pediatric ward because kids stick it in their mouth and it passes onto other kids
- c. Which toys are the worst fomites?
- i. STUFFED ANIMALS
- d. What are the best toys?
- . HARD PLASTIC → disinfect

FEASIBILITY

Can you do it? Could you actually play it with the child in the situation?

Example: Is swimming okay for a 13 year old? YES. More likely, common self.

AGE APPROPRIATE

1.) INFANCY → FROM 0-6 MONTHS OF AGE

- **Best toy: Musical mobile**
- These kids are sensory-motors
- Something that stimulates BOTH sensory and motor
- If boards does not have musical mobile, second best answer is: something SOFT but has to be LARGE → due to RF aspiration

2.) 6-9 MONTHS OF AGE

- Working on skill called OBJECT PERMANENCE
 - Idea that it's still there even though you can't see it
 - Example: If toy is under blanket, will they know that it's there?
- Your play should be teaching them that
- **Best toy: Cover-uncover toy**
- You cover it up, then you uncover it, repeat
- **Examples:** Jack in the box; pop up pals; books that have the little windows; peek a boo; cover yourself under the blanket
- **Second best toy: Something LARGE but FIRM**
 - Wood, metal, hard plastic

- **Worst toy: The musical mobile** → they pull themselves up and strangulation occurs

3.) 9-12 MONTHERS

- Working on vocalization
- **Speaking toys, talking toys, verbal toys**
- **Examples:** Woody the cowboy, tickle me elmo, talking books, talking teddy bear → they have a string or button then they say something
- Purposeful activity starts here
 - Could a 7 month old play with 3 wooden blocks? YES.
 - Could a 7 month old build a tower with 3 wooden blocks? NO.
 - “BUILD” is a purpose word
- Never pick answers with the following words if kid is <9 months:
 - Build
 - Sort
 - Stack
 - Make
 - Construct
- A baby is in the womb for... 9 months. And it's another 9 months until they can do something purposeful!

4.) TODDLERS: 1-3 YEARS

- **Best toy: Push or pull toy**
- **Examples:** Lawn mower, wagon, buggy, baby strollers, dog
- Working on gross-motor skills → running, jumping
- If it takes finger dexterity → NOT FOR TODDLER!
 - Coloured pencils? Scissors? Blunt scissors? NO.
 - Finger painting is not finger painting. It is hand painting. YES. because it takes your hand, not your finger.
- Parallel play → Play alongside but not with others

5.) PRESCHOOLERS

- Work on fine motor
- Finger dexterity
- Work on their balance
- **Examples:** Tricycles, dance class, ice skates, etc.
- Cooperative play → Play together and interact
- They like to pretend → Highly imaginative

6.) SCHOOL AGED → 3 C's

- **CREATIVE:** Let them make it. Don't give it to them.
 - Would it be good to give them a colouring book with pencils? NO. Give blank paper and pencils to create their own pictures.
 - Would you give them plastic lemonade? Plastic donuts? For a tea party? NO. They make their own stuff and have a tea party.
 - **Examples:** LEGO → build and make them stuff
- **COLLECTIVE:** always collecting something!!!
- **COMPETITIVE:** They like to play games where there's a winner or a loser
 - They like when there's a prize/winner/loser

7.) ADOLESCENTS

- Peer group association
- Do they do much? Not really. All of them just watch tv.
- If they gave you a question about a gang of teenagers hanging in a teenagers room, do you let them or not let them? You let them UNLESS...
 1. If anyone is fresh post-op (<12 hours out of surgery)
 2. If anyone is immunosuppressed
 3. If anyone has a contagious disease